

COGNITIVE BEHAVIORAL THERAPY (CBT) – SESSION 8

New Case: Aryan – Review & Beginning Relapse Prevention

Full Detailed – More Realistic Style

SESSION INFORMATION

Client:	Aryan (32 y/o Male)	Therapist:	Dr. A.
Session #:	8 of 18–20	Focus:	Review + Beginning Relapse Prevention

PRE-SESSION THERAPIST NOTES

Client brought the compassionate note to his younger self and found it emotionally meaningful. He had mixed results with Worry Postponement and behavioral activation. Belief in the old thought remains around 45–50% after the imagery work. Today's plan: Review homework, begin ****relapse prevention planning**** (early warning signs + coping strategies), continue cognitive work, and assign integrated homework.

SESSION START & HOMEWORK REVIEW

Therapist: Good morning, Aryan. How has your week been? I see you brought the compassionate note you wrote.

Client: It was average. I wrote the note to my younger self. It felt emotional but helpful. Worry Postponement helped on some days. I did two small activities again. The time-limit experiment was still hard. I'm still not seeing big changes, but the note made the new belief feel a bit more real.

Therapist: Thank you for sharing that. The fact that the compassionate note made the new belief feel more emotionally real is meaningful progress. These deeper shifts often happen gradually. Today we can start thinking about how to maintain the gains you've made so far.

BEGINNING RELAPSE PREVENTION PLANNING

Therapist: Since we're now at Session 8 (roughly the halfway point), I'd like to start preparing you for when therapy ends. One important part of CBT is having a clear plan to maintain your gains and handle difficult times in the future. This is called ****Relapse Prevention Planning****. We'll build it gradually over the next few sessions.

Client: That makes sense. I've been worried about what happens after we finish.

Therapist: That's completely normal. Let's start with the first step: identifying your ****Early Warning Signs**** — the signals that your anxiety or old thinking patterns might be returning.

Early Warning Signs we identified together:

- Increased time spent worrying during the day
- Returning to over-checking and staying up late
- Sleep starting to get disturbed again
- Low energy and motivation returning
- Irritability with wife increasing

TOP COPING STRATEGIES

Therapist: Next, let's list the strategies that have helped you so far. These will be your main tools when warning signs appear.

Top Coping Tools so far:

- Use the balanced thought and thought records
- Practice Worry Postponement
- Do small behavioral experiments when fear feels strong
- Schedule small activities even when motivation is low
- Talk to wife instead of withdrawing

HOMEWORK ASSIGNMENT

Therapist: For this week:

1. Continue daily diary + Worry Postponement practice
2. Continue scheduling 2–3 small activities per week
3. Continue the time-limit experiment with small tasks
4. Finalize and write down your personal Early Warning Signs list

SESSION SUMMARY & FEEDBACK

Therapist: Today we reviewed your meaningful work with the compassionate note, and we started building your relapse prevention plan by identifying early warning signs and top coping strategies. How does that feel?

Client: It feels good to start thinking about how to keep the progress after therapy ends. I'm still not seeing huge changes yet, but I'm willing to keep going.

Therapist: That's fair and realistic. Any questions?

Client: No.

POST-SESSION THERAPIST NOTES

- Client found the compassionate note emotionally meaningful — good progress.
- Started relapse prevention planning (Early Warning Signs + Top Coping Strategies).
- Belief in old thought remains around 45–50% after imagery work.
- Client continues to express realistic mixed feelings about progress.
- Next session: Review Early Warning Signs list, continue relapse prevention planning (Action Plan), deepen imagery or cognitive work.

Disclaimer: This is a completely fictional but realistic simulation created solely for educational and AI training purposes. It does not represent any real person or therapy.